Humana. Claims Payment Policy

Subject: Pass-Through Billing

Application: Medicare Advantage Products
Policy number: CP2015106
Related policies: N/A

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Overview

Pass-through billing occurs when a provider bills for a service for which neither that provider nor any individual under that provider's direct employment performed that service. One common example is that a provider ordered a service and bills for it, but an unaffiliated lab performed the service. Sub-contracted individuals are not under "direct employment."

Purpose

To facilitate compliance with Humana's state and federal obligations to ensure fair and accurate payment of health care claims for covered services, to avoid duplicate payments, overpayments and subsequent recoupments, to prevent fraud, waste and abuse, and to ensure accurate member copay, deductible and coinsurance obligations for members under their health benefit plan in compliance with all state and federal laws, rules and regulations.

Scope

This policy applies to all services to the extent that its application is not prohibited by applicable contract provisions and applicable state or federal law, rules and regulations including Centers for Medicare & Medicaid Services (CMS) instructions.

Medicare Advantage Payment Policy
Definitions of *Italicized* Terms
References
General Humana Resources

Medicare Advantage Payment Policy

In addition to the policy, claims payments are subject to other plan requirements for the processing and payment of claims, including, but not limited to, requirements of medical necessity and reasonableness and applicable referral or authorization requirements.

General

Pass-through billing is generally prohibited, with limited exceptions, for Medicare Advantage basic benefits (i.e., benefits that would be covered under Original Medicare Parts A and B). A provider may not bill, charge, seek payment or have any recourse against Humana or any Humana Medicare Advantage member for any amounts related to a denied pass-through billed service for basic benefits.

Limited Exceptions

The following are limited exceptions to the general prohibition against pass-through billing. A provider may submit a charge for potentially covered services rendered by another qualified provider under only the following circumstances. In all of these circumstances, the submitting provider may not bill an amount greater than the amount the submitting provider paid or will pay the rendering provider for the service. Humana will not pay any amount greater than the amount the submitting provider pays the rendering provider for the service. The billing provider must make available to Humana documentation of the amount paid to the rendering provider.

Laboratories billing for referred tests exception

A *referring laboratory* may bill for clinical laboratory diagnostic tests performed by a *reference laboratory* if one of the following conditions is met:

- The *referring laboratory* is located in, or is part of, a rural hospital; or
- The *referring laboratory* is wholly owned by the entity performing such tests, the *referring laboratory* wholly owns the entity performing such tests, or both the *referring laboratory* and the entity performing such tests are wholly-owned by a third entity; or
- The *referring laboratory* does not refer more than 30 percent of the clinical laboratory tests for which it receives requests during the year, not counting referrals made under the wholly-owned condition described above.

Purchased diagnostic or screening tests exception

Note: This section does not apply to clinical diagnostic laboratory tests paid by Original Medicare under the Clinical Laboratory Fee Schedule.

A physician or other professional may bill for the technical component (TC) and/or professional component (PC) of a diagnostic test that was ordered by the physician or other professional, or ordered by a party related to the billing physician or other supplier through common ownership or control. However, anti-markup payment limitations apply if the diagnostic test is performed by a physician who does not "share a practice" with the billing physician or other professional.

The performing provider for the TC is the professional who conducted or supervised the test, and the performing provider for the PC is the professional who interpreted the results.

Under anti-markup payment limitations, payment to the billing physician or other professional, minus beneficiary cost-sharing, for the TC or PC of the diagnostic test may not exceed the lowest of the following amounts:

- The performing provider's net charge, excluding any charges for equipment or space leased, to the billing physician or other professional;
- The billing physician's or other professional's actual charge; and
- The amount for the test that would be allowed if the performing provider billed directly.

Exception to the anti-markup payment limitation
If the performing physician is deemed to "share a practice" with the billing physician or other professional who ordered the test, the anti-markup payment limitation does not apply. A performing physician is considered to "share a practice" with the billing physician or other professional if either:

- The performing physician provides "substantially all" (at least 75 percent) of his or her professional services through the billing physician or other professional, as discussed below; or
- The services were performed under a "site of service/same building" arrangement as discussed below.

The "substantially all" services requirement will be satisfied if, at the time the billing physician or other professional submits a claim for a service provided by the performing physician, the billing physician or other professional has a reasonable belief that:

- For the 12 months prior to and including the month in which the service was performed, the performing physician provided substantially all of his or her professional services through the billing physician or other professional; or
- The performing physician will provide substantially all of his or her professional services through the billing physician or other professional for the next 12 months (including the month in which the service is performed).

The performing physician may be deemed to "share a practice" with the billing physician or other professional if the services were performed under a "site of service/same building" arrangement. This alternative approach requires the performing physician to be an owner, employer or independent contractor of the billing physician or other professional and requires that the TC or PC be performed in the "office of the billing physician or other supplier."

The "office of the billing physician or other supplier" is any medical office space, regardless of the number of locations, in which the ordering physician or other professional regularly provides patient care. It also includes space where the billing physician or other supplier furnishes diagnostic testing services, if the space is located in the "same building" in which the ordering physician or other ordering professional regularly provides patient care.

With respect to a billing physician or other professional that is a "physician organization," the "office of the billing physician or other supplier" is space in which the ordering physician provides substantially the full range of patient care services the ordering physician provides generally. "Same building" and "physician organization" are defined in 42 CFR §411.351.

The billing physician or other professional must keep on file the performing physician's name, address and the National Provider Identifier. The physician or other professional providing the TC or PC of the diagnostic test must be enrolled in the Medicare Program. No formal reassignment is necessary.

Ancillary service exception
A charge submitted by a Social Security Act Section 1861(u) "provider of services," for a service rendered by another provider that, under Original Medicare, would be payable to the Section 1861(u) "provider of services." A "provider of services" includes a hospital, critical access hospital, skilled nursing facility, comprehensive outpatient rehabilitation facility, home health agency, hospice program or, for certain purposes, a fund.

Other Medicare Program exceptions
Any other exceptions applicable to the Original Medicare Program.

Submission by the Rendering Provider

Claims that were denied as violating this pass-through billing policy when they were submitted by a referring provider may still be eligible for reimbursement when submitted by the provider who provided the services in question. If the provider who directly performed the service bills a clean claim for that service, Humana will adjudicate the claim for reimbursement pursuant to the benefits available under the member's health benefit plan.

Right to Recoup Reimbursement

If Humana discovers, through an audit or other means, that a provider has submitted a bill in violation of any of the requirements above, Humana reserves the right to recoup payments made to the provider pursuant to such bills.

Definitions of *Italicized* Terms

- ***Reference laboratory:*** The laboratory that performs the test on a specimen received from another referring laboratory.
- ***Referring laboratory:*** The laboratory that obtains a specimen and refers it to another laboratory for performance of the test.

References

- Social Security Administration website. Social Security Act. Title 11, Section 1128B – Criminal Penalties for Acts Involving Federal Health Care Programs. Paragraph (b). www.ssa.gov.
- Social Security Administration website. Social Security Act. Title 18, Section 1842 – Provisions Relating to the Administration of Part B. Paragraph (b)(6). www.ssa.gov.
- Social Security Administration website. Social Security Act. Title 18, Section 1861 – Definitions of Services, Institutions, etc. Paragraph (u) – Provider of Services. www.ssa.gov.
- U.S. Government Publishing Office website. United States Code. Title 42, Section 1395l – Payment of Benefits. Paragraph (h)(5)(A). www.govinfo.gov.
- U.S. Government Publishing Office website. United States Code. Title 42, Section 1395nn – Limitations on Certain Physician Referrals. www.govinfo.gov.
- U.S. Government Publishing Office website. Code of Federal Regulations. Title 42, Section 411.351 – Definitions. www.ecfr.gov.
- U.S. Government Publishing Office website. Code of Federal Regulations. Title 42, Section 422.500 – Scope and Definitions. Paragraph (a) – Scope. www.ecfr.gov.
- U.S. Government Publishing Office website. Code of Federal Regulations. Title 42, Section 1001.952 – Exceptions. www.ecfr.gov.
- Centers for Medicare & Medicaid Services website. Medicare Claims Processing Manual. Chapter 1 – General Billing Requirements. Sections 30.2, 30.2.1, 30.2.2, 30.2.2.1, 30.2.6, 30.2.7 and 30.2.9. www.cms.gov.
- Centers for Medicare & Medicaid Services website. Medicare Claims Processing Manual. Chapter 16 – Laboratory Services. Sections 40.1 and 40.1.1. www.cms.gov.
- Centers for Medicare & Medicaid Services website. Medicare Managed Care Manual. Chapter 4 – Benefits and Beneficiary Protections. Sections 10.2, 10.3 and 30.3. www.cms.gov.
- Centers for Medicare & Medicaid Services website. Medicare Learning Network. MLN Booklet: Medicare Fraud & Abuse: Prevent, Detect, Report. www.cms.gov.
- Centers for Medicare & Medicaid services website. Transmittal 445, Change Request 6371 – Claims Processing Instructions for Diagnostic Tests Subject to the Anti-Markup Pricing Limitation. www.cms.gov.
- Centers for Medicare & Medicaid Services website. Transmittal 3103, Change Request 8806 – Reporting the Service Location National Provider Identifier (NPI) on Anti-Markup and Reference Laboratory Claims. Section (I)(B) – Policy. www.cms.gov.
- Humana website. Provider Manual for Physicians, Hospitals and Healthcare Providers. Humana.com.

General Humana Resources

- Availity – Providers can register for access to information on a variety of topics such as eligibility, benefits, referrals, authorizations, claims and electronic remittances.
- Claims processing edit notifications – Alerts of upcoming claims payment changes are posted on the first Friday of each month.
- Claims resources – Providers can find information on referrals, authorizations, electronic claim submissions and more.
- Making it easier – This page contains an educational series for providers and healthcare professionals.
- Medical and pharmacy coverage policies – Humana publishes determinations of coverage of medical procedures, devices and medications for the treatment of various conditions. There may be variances in coverage among plans.

- Publications – This page can help you find our quarterly newsletter, provider manual and other resources to help you do business with us.

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